



SURGICAL PATHOLOGY REPORT

Patient Name:

Med. Rec. #:

DOB/Gender:

Location:

Soc. Sec. #:

Physician(s):

Path No.:

Client:

Collected:

Received:

Reported:

Hosp #:

Pre-Operative/Clinical History

Left breast cancer

Specimen(s) Received

- A: LEFT AXILLA SENTINEL LYMPH NODE #1
B: LEFT AXILLA SENTINEL LYMPH NODE #2
C: LEFT AXILLA, ADDITIONAL AXILLARY TISSUE
D: BREAST, MASTECTOMY

1CB-0-3

Carcinoma, infiltrating lobular, NOS 8520/3
Site: breast, NOS C50.9 *fw 12/2/11*

Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
HPAA Discrepancy		X
Prior Malignancy History		X
Qual/Synchronous Primary Noted		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	fw 12/2/11	12/2/11

Gross Description

Specimen A, received in formalin labeled left axilla sentinel node #1, consists of adipose tissue, 3.5 x 1.5 x 1.3 cm. Sections show a lymph node measuring 2.0 cm in greatest dimension, partially replaced by adipose tissue. The node is sectioned and entirely submitted labeled (A1, A2).

Specimen B, received in formalin labeled left axilla sentinel node #2, consists of adipose tissue, 2.0 x 1.5 x 0.8 cm. Sections show a single lymph node candidate measuring 1.0 cm in greatest dimension. The node is sectioned and entirely submitted. (B1).

Specimen C, received in formalin labeled left axilla additional axillary tissue, consists of adipose tissue, 4.0 x 2.5 x 2.0 cm in aggregate. Sections show a single lymph node candidate, 0.3 cm in greatest dimension. (C1).

Microscopic Description

The microscopic findings are reflected in the diagnosis rendered.

Diagnosis

- A) LYMPH NODE, LEFT AXILLARY SENTINEL #1, EXCISION:
ONE LYMPH NODE INVOLVED BY METASTATIC MAMMARY CARCINOMA (9 MM, NEGATIVE FOR EXTRACAPSULAR EXTENSION).
- B) LYMPH NODE, LEFT AXILLARY SENTINEL #2, EXCISION:
ONE LYMPH NODE INVOLVED BY MICROMETASTATIC MAMMARY CARCINOMA (1 MM, NEGATIVE FOR EXTRACAPSULAR EXTENSION)..
- C) LYMPH NODE, LEFT AXILLARY NONSENTINEL, EXCISION:
ONE LYMPH NODE NEGATIVE FOR TUMOR.

PHD HISTOLOGY REPORT

Electronically Reviewed and Signed Out By

Comment

All sentinel node blocks were examined with three 80 um interval H&E stained sections and two 80 um interval cytokeratin AE1/AE3 immunostains.

Positive and negative controls react satisfactorily.

FDA required disclaimer: These tests were developed and their performance characteristics determined by [redacted] They have not been cleared or approved by the U.S. Food and Drug Administration. The FDA has determined that such clearance or approval is not necessary. These tests are used for clinical purposes and should not be regarded as investigational or for research. This laboratory is certified under the Clinical Improvement Amendments of 1988 (CLIA) as qualified to perform high complexity clinical laboratory testing.

acquired information for predictive/prognostic markers: 1. Type of specimen fixation and detection system: Tissue is fixed in 10% formalin. Indirect biotin-free detection kit is used. 2. Clones used: ER (clone 6F11) Novocastra; PR (clone PGR 636) DakoCytomation; CD117 (polyclonal, DakoCytomation); HER2 (clone 4B5, Ventana, FDA approved); Ki-67 (clone K3, Cell Marque) p53 (clone DO7, Novocastra), MSH6 (clone BC-44, Biocare), MSH2 (CellMarque clone G219-1129), MLH1 (Cell Marque clone G168-728), PMS2 (clone MRQ-28, Cell Marque). 3. Criteria for positive results: ER and PR: $\geq 1\%$ of tumor cells with nuclear staining; HER2: $>30\%$ of cells show complete membrane staining; p53: $\geq 10\%$ of tumor cells with nuclear staining; MMR Proteins: complete absence of tumor nuclear staining.

CPT Code(s)

A: 88307, 88342,

B: 88307, 88342,

C: 88305

D: 88307, 3260F

Procedures/Addenda

ADDENDUM

Addendum Comment

Gross Description

Specimen D is received in formalin labeled left breast, suture = axillary tail.

Specimen type:	Mastectomy without axillary tail and without pectoralis muscle. The specimen is oriented by the surgeon as follows: suture = axillary tail.
Specimen size/description:	755 gm. SI 17.5 cm, ML 16.5 cm, AP 6.0 cm.
Laterality:	Left.
Tumor size:	3.0 x 2.5 x 2.0 cm.
Tumor site:	Upper inner quadrant, 10-11 o'clock.
Tumor description:	Stellate, gritty, tan-white with a central 1.0 x 1.0 x 0.7 cm hemorrhagic ill-defined biopsy site.
Deep margin:	1.5 cm.
Nipple/areolar complex:	The areola measures 3.5 cm with a centrally retracted 0.6 x 0.6 cm nipple. There is a 5.5 x 4.0 cm area of blue dye superior the nipple/areolar area.

PHD HISTOLOGY REPORT

Skin: The ellipse measures 22.0 x 10.5 cm.

Lymph node sampling: Not applicable.

Other: Sectioning of the surrounding breast tissue shows predominantly yellow lobular adipose tissue admixed with dense, granular cystic breast tissue.

Sections are submitted as follows:

(D1, 2) Nipple
(D3-7) Mass
(D8, 9) Upper inner quadrant
(D10, 11) Lower inner quadrant
(D12, 13) Lower outer quadrant
(D14, 15) Upper outer quadrant
(D16, 17) Deep margin.
EO:vyb

****Electronically Reviewed and Signed Out By*

ADDENDUM

Addendum Diagnosis

D) BREAST, LEFT, TOTAL MASTECTOMY:

INVASIVE LOBULAR CARCINOMA, 3.0 CM, LOW COMBINED HISTOLOGIC GRADE WITH LOW PROLIFERATIVE RATE.

SURGICAL MARGINS NEGATIVE FOR TUMOR (SEE COMMENT).

Addendum Comment

**Summary of Pathologic Findings
Breast Carcinoma**

Microscopic Examination

Specimen: Total breast.

Procedure: Total mastectomy.

Lymph node sampling: Sentinel node biopsies.

Specimen Integrity: Single intact specimen.

Specimen size: 17.5 cm greatest dimension.

Laterality: Left.

Tumor site: Upper inner quadrant, 9-10 o'clock position per previous diagnostic core biopsy.

Size of invasive component: 3.0 cm greatest dimension.

Tumor focality: Single focus.

PHD HISTOLOGY REPORT

Macroscopic and microscopic extent of tumor:

Skin: Uninvolved.
Nipple: Paget disease absent.
Skeletal muscle: Not present.

Histologic type: Invasive lobular carcinoma.

Combined histologic grade: Low combined histologic grade with low proliferative rate.

Tubule formation: 3.
Nuclear pleomorphism: 1.
Mitotic count: 1 (field area 0.23 mm²).

Total Nottingham score: 5.

Lymph-vascular invasion: Absent.

Ductal carcinoma in situ: Absent.

Extensive intraductal component (EIC): Not applicable.
Extent: Not applicable.

Lobular carcinoma in situ: Absent. There are focal areas of atypical lobular hyperplasia.

Deep margin: Not involved.

Lymph nodes:

Number of sentinel nodes examined: 2.
Total number of nodes examined: 3.
Number of nodes with macrometastasis: 1.
Number of nodes with micrometastasis: 1.
Number of nodes with isolated tumor cells: 0.

Size of largest metastatic deposit: 9 mm.

Extracapsular extension: Negative.

Method of evaluation of sentinel lymph nodes:

Three 80 μ m interval H&E stained sections and two 80 μ m interval cytokeratin AE1/AE3 immunohistochemical stains on all sentinel lymph node blocks.

TNM stage: pT2 N1a.

Ancillary studies (estrogen receptor, progesterone receptor, HER2/neu and Ki-67):

Performed on previous specimen, see report for results.

***Electronically Reviewed and Signed Out By

ADDENDUM

PHD HISTOLOGY REPORT

Addendum Comment

Total fixation time in formalin: 31.0 hours.

****Electronically Reviewed and Signed Out By*

END OF REPORT

Page 5 of 5